

ENCOUNTER 1 OF 5

Date of Service: 2024-06-15 | Type: OUTPATIENT VISIT | Specialty: Orthopedic Surgery | Setting: Outpatient | Acuity: LOW

Attending Provider:	Dr. Anita Reyes, MD	Location:	New Amandaville Orthopedic & Sports Medicine Clinic
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Chief Complaint

Numbness and tingling in the left toes, present for approximately 6 months.

History of Present Illness

Ms. Gardner is a 44-year-old female who presents for initial evaluation of numbness and tingling in the left toes with occasional radiation into the left foot. She describes the onset as gradual, beginning roughly six months prior to this visit, with a constant pins-and-needles quality that does not fully resolve throughout the day. She rates the discomfort at 3/10 in severity. Symptoms are aggravated by prolonged standing or walking, which is notable given the standing/walking demands of her occupation, and are partially relieved with rest and elevation of the left leg. She denies any preceding trauma, recent falls, or new footwear. She has not previously sought care for this specific complaint but has self-treated with over-the-counter NSAIDs and acetaminophen with minimal relief. She denies bowel or bladder dysfunction, denies bilateral symptoms, and denies fever or systemic illness. She reports mild intermittent swelling of the left foot and occasional shooting pains radiating up the left leg, but denies redness, warmth, or open wounds. Given the chronicity and functional impact on her occupation, she was referred to orthopedic surgery by her primary care provider for further work-up.

She denies any history of diabetes mellitus, though given her constellation of symptoms a metabolic and structural work-up is indicated to rule out peripheral neuropathy from endocrine, compressive, or structural etiologies. She has not had any prior imaging of the foot or ankle. She is independently ambulatory without assistive devices and reports no falls related to the numbness.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
HEENT	History of seasonal allergies; denies vision changes or hearing loss.
Cardiovascular	History of hypertension, well controlled; denies chest pain or palpitations.
Respiratory	History of mild intermittent asthma; denies current dyspnea or wheeze.
Gastrointestinal	Reports occasional constipation; denies nausea, vomiting, or GI bleeding.
Genitourinary	Reports occasional urinary frequency; denies dysuria or hematuria.
Musculoskeletal	Reports mild left knee stiffness; denies acute joint swelling elsewhere.
Neurological	Reports numbness/tingling in left toes; denies headache, seizure, or syncope.
Psychiatric	History of generalized anxiety, stable on no pharmacotherapy; denies SI/HI.
Endocrine	History of hypothyroidism, on levothyroxine; denies heat/cold intolerance.
Integumentary	Reports occasional dry skin; denies rash or new lesions.
Hematologic/Lymphatic	History of mild anemia; denies easy bruising or lymphadenopathy.

Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

Social History

The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	128/72 mmHg	Heart Rate	68 bpm
Temperature	98.2 F	Respiratory Rate	16 breaths/min
Pain Score	3/10	SpO2	97% RA
Weight	156 lbs	Height	5 ft 5 in
BMI	26.0 kg/m2		

Physical Examination

General

Well-developed, well-nourished female in no acute distress, ambulating with a normal, non-antalgic gait.

Cardiovascular

Regular rate and rhythm, S1/S2 normal, no murmurs, gallops, or rubs. No jugular venous distension. 2+ pedal pulses bilaterally.

Respiratory

Clear to auscultation bilaterally, no wheezes, rhonchi, or crackles. Symmetric chest wall expansion.

Musculoskeletal

Left foot with mild dorsal swelling, no erythema or warmth. No palpable mass appreciated on this initial exam. Mild stiffness of the left knee with full, painless range of motion. Full range of motion of the left hip and ankle. Tinel's sign equivocal at the level of the tarsal tunnel.

Neurological

Intact sensation to light touch and pinprick in the right lower extremity. Decreased sensation to light touch and pinprick over the dorsal aspect of the left toes and forefoot. 2+ deep tendon reflexes bilaterally at the patella and Achilles. Motor strength 5/5 throughout bilateral lower extremities. No clonus.

Skin

No rash, ulceration, or skin breakdown noted on either lower extremity.

Diagnostic Studies Ordered

Test	Result	Reference Range	Flag
Complete Blood Count (CBC)	Pending	See reference lab	-
Comprehensive Metabolic Panel (CMP)	Pending	See reference lab	-
Hemoglobin A1c	Pending	< 5.7 %	-
TSH	Pending	0.4 - 4.0 uIU/mL	-
X-ray, Left Foot (3 views)	Pending	N/A	-

Assessment

1. Left forefoot/toe paresthesia and numbness, chronic, of uncertain etiology; differential includes tarsal tunnel syndrome, Morton's neuroma, peripheral neuropathy (metabolic vs. compressive), and early degenerative changes of the left foot.
2. Hypertension, controlled on current regimen.
3. Mild intermittent asthma, currently well controlled.
4. Primary hypothyroidism, on stable levothyroxine dosing.

Plan

1. Obtain CBC, CMP, hemoglobin A1c, and TSH to screen for metabolic causes of peripheral neuropathy.
2. Obtain 3-view weight-bearing X-ray of the left foot to evaluate for structural or osseous abnormality.

3. Trial of a course of formal physical therapy focused on gait mechanics, stretching, and nerve gliding exercises, 2x weekly for 4 weeks.
4. Continue current home medications without change at this time.
5. Recommend supportive, cushioned footwear and activity modification to minimize prolonged standing where feasible.
6. Return to clinic in approximately 8 weeks to review labs, imaging, and response to physical therapy, sooner if symptoms acutely worsen or if she develops weakness, bowel/bladder changes, or bilateral symptoms.

Patient Education / Instructions

The patient was counseled on the benign nature of the working differential at this stage, the rationale for laboratory and imaging work-up, and expected physical therapy course. She was advised to seek urgent care if she develops acute weakness, loss of bladder or bowel control, rapidly progressive symptoms, or signs of infection (fever, spreading redness, warmth) in the affected extremity. She verbalized understanding and agreement with the plan.

Care Team

Name	Role	Contact
Anita Reyes, MD	Attending Orthopedic Surgeon	555-201-4410
Daniel Ho, PA-C	Orthopedic Physician Assistant	555-201-4415
Karen Ibbotson, RN	Clinic Nurse	555-201-4420
Dr. Susan Whitfield, MD	Primary Care Physician (referring)	555-201-3300

Pain & Functional Assessment Questionnaire

Question	Response
Average pain over past week (0-10 scale)	3/10
Worst pain over past week (0-10 scale)	5/10
Does pain interfere with walking?	Mild interference
Does pain interfere with sleep?	No
Does pain interfere with work duties?	Mild interference
Any assistive devices used?	None
Overall satisfaction with current function (0-10)	6/10

Care Coordination Note

Referral to physical therapy submitted and authorized through the patient's Kaiser plan; the patient was provided with a list of in-network physical therapy providers near her home in New Amandaville and elected to schedule with Foothill Physical Therapy. Laboratory orders were transmitted electronically to the reference laboratory, and the patient was instructed to complete blood draws within the next 5-7 days. Radiology order for left foot X-ray was scheduled same-day in the outpatient imaging suite following this visit. The clinic nurse reviewed medication reconciliation with the patient, confirming no changes to her home medication list since her last primary care visit. The patient's primary care physician, Dr. Susan Whitfield, was copied on this consultation note for continuity of care. No durable medical equipment was required at this time. The patient was provided clinic contact information and after-hours triage line information in case of worsening symptoms.

Electronically signed by: **Anita Reyes, MD**
Department: Orthopedic Surgery
Signed: 2024-06-15 4:42 PM